

**CANADIAN RAILWAY OFFICE OF ARBITRATION
& DISPUTE RESOLUTION**

CASE NO. 5019-S

Heard in Montreal, October 8, 2024

Concerning

CANADIAN PACIFIC KANSAS CITY RAILWAY

And

TEAMSTERS CANADA RAIL CONFERENCE

DISPUTE:

The compensation owed to Mr. JB ("the Grievor") following CROA 5019.

JOINT STATEMENT OF ISSUE:

On March 14, 2024, the parties presented their positions regarding the Grievor's Appeal of the Company's new Return to Work Agreement ("RTWA" or the "Agreement") provided to JB ("Grievor") on January 10, 2022 (version dated October 21, 2021) and the impact that this had on the Grievor's return to the workplace including lost wages and other entitlements.

Arbitrator Cameron issued award CROA 5019 on June 4, 2024, which outlined the following:

Position of the Parties

40. The Union seeks full compensation for the grievor between January 10, 2022 and April 14, 2023.
41. The Company asks that the matter be remitted to the Parties, noting outstanding factual issues as to when the grievor was available to work.
42. The Union objects to the raising of these factual issues, but argues that the grievor was available to work and deserves full compensation.

Analysis and decision

43. I find that the grievor is entitled to full compensation for all periods he was available to work after January 10, 2022 until April 14, 2023.
44. The matter is remitted to the Parties to discuss implementation of the remedy.
45. I remain seized with respect to any questions of interpretation or application of this Award, as well as with respect to remedy, should the Parties be unable to reach a solution.

The parties were not able to reach agreement on the amount of compensation owed to the Grievor including the period(s) of time in which the grievor was entitled to compensation.

As the Arbitrator remains seized, the parties request for a determination on the periods of time that the grievor would be entitled to compensation as well as what the compensation should be based on, as this will most likely be in contention as well.

UNION'S POSITION:

The Union's position is that the Company's violation of the requirement for JB to sign their new Return to Work Agreement ("RTWA" or the "Agreement") provided on January 10, 2022 (version dated October 21, 2021), impacted the Grievor's return to the workplace at all times.

JB provided a FAF on January 27, 2022 clearing him for immediate return to work with no restrictions. It was the Company's new RTWA that prevented JB from returning in any aspect thus he has been financially impacted by the Company's violation, and must be compensated for the entire period he was held from service.

Once the Company changed the RTWA again (as provide in the Union's briefs previously) and removed the offensive language, it was eventually provided (no delay of JB) to JB who then signed it. From there JB was only required to now provide a periodic medical that had expired and return to duty testing. At all times the Company could have had this requested to be completed.

The Union request that JB be compensated all loss of wages from the noted period of time previously provided and that JB be compensated at his CTY rate of pay for that period without any mitigation. The Company at all times could have returned JB to his safety critical position and/or a proper accommodation based on the fact that his doctor provided no restrictions for his return. It was the Company's delay account of the unsigned RTWA, again it must be repeated, once the re-drafted RTWA was provided to JB, it was signed and he need only present himself for his periodic medical and return to duty testing, no other documentation was requested by CP (OHS) once this was done.

No accommodation position was provided.

The Company's Position:

The Company disagrees with the Union's positions and requested remedy.

The Company acknowledges that based on the CROA 5019, the Grievor is entitled to some compensation but not to the extent sought by the Union.

Throughout the management of the Grievor's situation, the Company's actions were aligned with its legislative and regulatory requirements to ensure a safe workplace.

Despite the Grievor not signing the RTWA (version dated October 21, 2021) the Company continuously engaged with the Grievor throughout the Fitness to Work process by seeking medical information to determine his fitness for duty for his regular Safety Critical Position.

Based on the medical information, the Grievor was only fit for modified duties during the period of January 10, 2022 to April 14, 2023. The Grievor was 'Fit for non-safety sensitive duties' and is therefore not entitled to compensation at the rate of his regular duties. Any compensation should be a at a reduced rate.

The Grievor's medical restrictions were based on medical evidence and determined by the Office of the Chief Medical Officer - not a failure to sign the RTWA as alleged by the Union.

During the period of January 13, 2022 through to September 28, 2022 the Grievor should not be entitled to any compensation as he was found to be non-compliant with the Company's Fitness to Work Medical Policy & Procedures which delayed the Company's ability to assess if there were any changes to the Grievor's medical restrictions and the determination of his fitness for duty.

Additionally, throughout the period in dispute the Company sought accommodated positions.

For the Union:
(SGD.) W. Apsey
 General Chairperson

For the Company:
(SGD.) F. Billings
 Director, Labour Relations

There appeared on behalf of the Company:

F. Billings	- Director, Labour Relations, Calgary
S. Oliver	- Manager, Labour Relations, Calgary

And on behalf of the Union:

M. Church	- Counsel, Caley Wray, Toronto
D. Psychogios	- General Chairperson, CTY-E, Montreal
J. Diggles	- Vice General Chairperson, CTY-E, Video Conferencing
J.B.	- Grievor

AWARD OF THE ARBITRATOR

Context and Issues

1. This matter arises out of dispute concerning the appropriate compensation to be paid to JB, following the decision in **CROA 5019**. The time period in issue is between January 10, 2022 and April 14, 2023.

2. With respect to this period, the Parties disagree whether the grievor was able to do Safety Critical work, or only non-Safety-Critical, non-Safety-Sensitive work at a reduced rate of pay. They also disagree whether the Company has met its obligations to properly accommodate the grievor. They further disagree whether the grievor should be disentitled to any pay during a period of alleged non-compliance, between January 13, 2022 and September 28, 2022. The Company does admit that the grievor is entitled to pay at a reduced rate between January 1, 2023 and April 14, 2023.

3. The facts are largely not in dispute; it is more a question of characterization and weight to be given to particular facts. Both Parties accept that the railway industry is a dangerous one, and employees must be Fit to Work. This is particularly important in the case of Safety Critical jobs, such as Conductor, held by the grievor, but also true for Safety Sensitive jobs.

4. The Company has emphasized the importance of the Canadian Railway Medical Rules Handbook (see Tab 3, Company documents). The Rules define the role of the Chief Medical Officer (s. 3), and how substance abuse disorders are to be addressed (s. 4.8).

5. The Union has emphasized that the grievor self-reported his alcohol issues, sought treatment and was cleared by his treating psychiatrist for return to a Safety Critical role in January 2022. The Union relies on arbitral jurisprudence confirming that when a medical note is produced by an employee finding the individual able to return to work, the Company bears the onus should it choose to contest the note (see **Fort James Canada and GCIU Local 100-M**, 2001 CarswellOnt 5998, **Nutrien and USW, Local 3552**, 2024 CarswellSask 35). It argues that the Company has not met this onus.

6. The Issues between the Parties are as follows:

- A.** At what time did JB become capable of performing Safety Critical work?
- B.** At what time after January 10, 2022 and before August 15, 2022 was JB capable of performing non-Safety Critical, non-Safety Sensitive work?
- C.** Did the Company meet its duty to accommodate JB?
- D.** Is JB disentitled to pay between January 13, 2022 and September 28, 2022 due to non-compliance?

A. At what time did JB become capable of performing Safety Critical work?

Position of the Parties

7. The Company takes the position that the grievor was not capable of performing Safety Critical work prior to April 14, 2023, based on Fitness to Work Assessments and the restrictions put in place under the authority of the Chief Medical Officer.

8. The Company argues that it was obliged to obtain further information to clarify ambiguities arising from the assessments done.

9. The Union argues that the grievor should have been permitted to go back to his work as a Conductor, based on the January 2022 reports from his treating psychiatrist, Dr. Rabinovitch, that his alcohol abuse problems were under control.

10. The Union further notes that the grievor was sent by the Company to an addiction medicine specialist, Dr. Lucknow, whose July 2022 report also cleared the grievor for a return to a safety critical role as Conductor.

11. It submits that the additional information sought by the Company should not have prevented the grievor from returning to work as a Conductor.

Analysis and Decision

12. For the reasons that follow, I find that the grievor was capable of performing Safety Critical work as a Conductor by August 15, 2022.

13. A Functional Abilities Form clearing the grievor for Safety Critical work was signed January 25, 2022 by the grievor's treating psychiatrist, Dr. Rabinovitch. The Form provided by the Company specifically notifies the treatment provider of the nature of Safety Critical work and the importance of the FAF:

Part 3 Information for the Treatment Provider

Your patient occupies either a Safety Critical Position (e.g. Locomotive Engineer, Conductor or Rail Traffic Controller) and operates or controls the movement of trains or a safety Critical Aviation Position (e.g. Pilot) and controls or operates aircraft. Impaired performance due to a medical condition could result in a significant incident affecting the health and safety of employees, the public, property or the environment. As a Physician assessing persons occupying these positions you are responsible under the Railway Safety Act to notify the Railway Company's Chief Medical Officer or notify the Regional Aviation Medical Officer under the Aeronautic Act if an employee has a medical condition that could be a threat to safe railway or flight operations.

1. Complete the attached FAF during the appointment.
2. Fax the completed form and invoice to Health Services
3. Provide a copy of the completed FAF to your patient.

Please Be Advised, That You Must Also Report If Your Patient Has Any Of The Following Medical Condition(S) Or Any Condition Which May Post A Threat To Safe Railway Or Aviation Operations.

Significant Hearing or Vision Deficit	Substance Use Disorder
Epileptic Seizure	Diabetes
Mental Health Disorder	Severe Sleep Apnea

Cardiovascular Disorder

Opioid Pain Medication Use

AND/OR any other medical condition(s) or medication(s) which may pose a threat to safe railway or flight operations.

14. The completed FAF notes the following:

- Diagnosis: **A) Alcohol Abuse (reports complete abstinence x 3 months)**
- **B) Cluster B personality traits**
- Rehabilitation Program: **Fosters Pavilion November 1-ongoing**
- In your opinion, is your patient at risk for sudden incapacitation or sudden impairment? **No At this time he is stable**
- Due to injury or illness this employee is: Fit for Regular Hours and Duties immediately, or as of: **Now**
- Complete Recovery Expected: **Yes**
- Estimated date for full RTW: **Now**
- Driving Vehicles Fit to Drive: **Personal vehicle, Company vehicle without passengers (car, pickup truck), Company vehicle with passengers**
- Operating Moving Equipment: **Fit and able to operate moving equipment**
- Effects on Cognition Due to medications and/or medical condition: Alertness, Attention, Orientation, Judgment, Memory, Mood, Psychomotor Functions: **No**
- Othe Limitations/Abilities/Comments: **Currently none**

15. Dr. Rabinovitch also signed a Morneau Shepell Form to the same effect (see Tab 16, Union documents).

16. The grievor had been off work as a result of a self-disclosed alcohol issue. These documents, coming from his treating psychiatrist, certainly qualify as prima facie evidence that the grievor was fit to work and able to return to a safety critical role as Conductor. The Union has cited a number of cases showing that a doctor's note establishes this prima facie evidence of fitness and that the Company then bears the burden of proof should it wish to contest this evidence. In the **Fort James Canada** matter, Arbitrator Newman held as follows:

The Union's reference to authority begins with Brown & Beatty, at para. 8:3342, which states, in part, at p. 8-92:

Proof of recovery. Fundamental to an employee's right to return to gainful employment is a determination that she is

capable of performing the job for which she has applied. When an employee returns from a period of illness or injury claiming that she is fit to return to work, she will bear an initial onus of substantiating that assertion. In the usual case, this onus will be satisfied by the employee presenting a valid medical certificate of fitness. However, in such circumstances, most arbitrators have recognized that the employer has an independent right, and according to some a duty, to satisfy itself that the employee is medically fit. The right is not an absolute and unfettered one. Rather, it has been stipulated that the right is premised on the employer having reasonable and probable grounds for assuming that the employee is unfit or would present a danger to himself, his fellow employees, or to company property.

And, at p. 8-92.1 (8-93 July 2000):

In all of these circumstances where the parties are unable to resolve the status of an employee's state of health, it ultimately will fall to the arbitrator to make a finding as to whether the employer's refusal to permit the employee to return to work was reasonable.

Characterizing the issue in such a manner, it would seem to follow that the employer must bear the ultimate burden of proving that its decision not to allow the employee to return to work was reasonable and *bona fide*.

I take the Brown & Beatty approach to be that the initial onus to demonstrate fitness to return to work is upon the returning employee. At arbitration the onus falls to the Union. That onus will usually be satisfied with production of a doctor's note. The onus will then shift to the Company to demonstrate reasonable grounds for not returning the employee to work at that time.

This is the approach followed in the *Toronto (City)* case cited by the Union, and more recently, in the *Nelson Laundries* award. The doctor's note, in each case, was taken as demonstrating fitness to return to work. If the Company had grounds upon which to refuse to return the grievor to work, in effect terminating the employment, the onus was found to rest upon it to demonstrate the ground for that action." (underlining added)

17. The Company chose to verify the report of Dr. Rabinovitch, by addressing numerous questions to an addiction medical specialist. Dr. Luckow provided his 11-page

detailed report to the Company in late July 2022 (see Tab 27, Union documents). The report answers 13 detailed questions from the Company:

3. Is this individual currently abstinent from alcohol and prohibited substances, and if so, what was the date of last usage?

Mr. B tells me his last alcoholic drink was on September 18, 2021. - He tells me he last smoked cannabis 9 years ago, but "may" have smoked some cannabis since then. He confirms, though, his last consumption was at least 5 years ago. He last used cocaine 10 years ago.

4. A current in DSM-IV diagnostic opinion – specifically, does this individual meet the diagnostic criteria for substance abuse and/or substance dependence? If so, for what substances?

- Mr. B. meets the criteria of alcohol dependence, in early remission (10 months sober). While I read in my mandate that he has a diagnosis of moderate cannabis use disorder, the information provided by Mr. B does not allow me to confirm this diagnosis. If new information is provided to me, a complimentary report can be produced.

5. If applicable, is this individual in active and stable recovery?

- Mr. B. consults AA meetings weekly. He also has remained sober of alcohol for 10 months. I consider that he is in active and stable recovery.

7. Recommendations regarding any further investigations and inpatient/outpatient treatment.

- There is no medical indication to recommend inpatient treatment. I recommend that he continues to participate in AA meetings, weekly. I also recommend that his physician consider the use of naltrexone, a medication that has been shown to reduce the risk of relapse of alcohol use.

8. Recommendations for follow-up and monitoring.

- Given the risk of relapse, I recommend ongoing monitoring for alcohol use. I recommend hair testing, to be done every three months, to measure hair for ethyl glucuronide.

9. An opinion on the likelihood of sustained abstinence. This opinion should be supported by the best available evidence-based literature.

- As mentioned above, a study done in 2006 showed there is a 62% chance of sustained remission at 3 years of sobriety, for someone who has received treatment, such as for Mr. B.

11. An opinion on individual's fitness to work in a Safety Critical Position. Specifically, any concerns with alertness, attention, concentration, judgement, or orientation related to the diagnosis or medications must be reported.

- Mr. B. denies having any cognitive or concentration issues. He tells me he has been sober from alcohol for 10 months. He has not consumed cannabis for at least 5 years, if not longer. He has demonstrated reasonable judgement over the past 10 months while obtaining medical care for his alcohol disorder. I find no medical reason that Mr. B cannot work in Safety Critical Position.

My only concern relates to his use of quetiapine, a medication with a potential to cause sedation. However, Mr. B does not report having this side effect, and its ongoing use could be medically beneficial. I recommend that his psychiatrist comment on whether ongoing use of quetiapine is medically necessary, whether its use presents a safety risk.

12. If you are of the opinion that the individual is to work in a Safety Critical Position, please provide the rationale for this opinion. Also specify the steps that must be taken by this individual, and the length of time these steps must be observed (e.g. two years), in order to reasonably demonstrate that the risk of relapse has so significantly reduced such that a return to Safety Critical Duties may be considered.

- As described in my answer to question 11, I believe that Mr. B is fit to work in a Safety Critical Position. As described in my question 7, I recommend that his physician consider the use of naltrexone to reduce the risk of relapse of alcohol. As described in my answer to question 8, I recommend ongoing monitoring of hair for ethyl glucuronide. I recommend monitoring for at least 2 years.
(underlining added)

18. As part of his evaluation, Dr. Luckow also had the grievor's hair sample tested for a drug panel, including cocaine and cannabinoids. The results were entirely negative (see Tab 28, Union documents).

19. The Company does not appear to directly contest the reports of Drs. Rabinovitch and Luckow. However, both internally and as expressed in communications with the

grievor, there were concerns about apparent contradictions in information given by the grievor to the medical profession and to the Company. The most serious contradiction appears to be the fact that the grievor did not disclose on his hiring that he had used cocaine some ten years ago, well prior to his hiring by the Company in 2018. There were also concerns about a gambling issue which had resulted in a two week hospitalization, some eight years ago. Finally, additional information was sought about an emergency room visit made in 2021 concerning his alcohol issues. These issues took months to resolve. Ultimately, the issues were resolved by the signing of the amended Return to Work Agreement, which was the subject of **CROA 5019**.

20. It is clear that the Company bears the burden of proof to establish why the Return to Work note signed by the grievor's doctor was insufficient or not to be relied upon. If the Company fails to meet this burden, the original medical note must be given effect. In this matter, the Company clearly had multiple concerns about the sufficiency of the report of Dr. Rabinovitch. They selected an addictions medicine specialist, Dr. Lucknow, who provided an extensive report which cleared the grievor for a return to a Safety Critical role as a Conductor.

21. While I give appropriate deference to the role and opinion of the Chief Medical Officer, it is the Company which must overcome the presumption that the medical note from Dr. Rabinovitch should be acted upon. I find that the concerns held by the CMO should have been met by the in-depth report from Dr. Lucknow. The remaining concerns do not appear to be sufficient to overcome the initial presumption, as supported by the report from an addiction medicine specialist. Dr. Lucknow was aware of the cocaine use some ten (10) years prior and nonetheless gave his opinion that the grievor was able to Return to Work. The gambling issue undoubtedly negatively affected the grievor, but there is no explanation from the Company why it should warrant safety concerns from the Company. The emergency room visit from 2021 was well known to Dr. Rabinovitch and did not prevent him from finding that the grievor was ready to return to a Safety Critical role.

22. While I acknowledge that the CMO had lingering concerns about the grievor, the Company has not convinced me that these concerns warrant keeping the grievor from a return to work after the reports of Drs. Rabinovitch and Lucknow. Accordingly, I find that the grievor should have been permitted to return to a Safety Critical role by August 15, 2022, following the receipt of the report from Dr. Lucknow in July, and having time to evaluate it.

23. The Parties have argued strenuously about the role of the amended RTWA, the subject of **CROA 5019**. The Union alleges that the delay in the return to work of the grievor is due solely to this dispute, ultimately decided in favour of the Union. The Company argues that these were two parallel processes, with the negotiations over the RTWA not impeding the extensive medical review of the grievor's file, and argues that it is the medical component which prevented an earlier return to work.

24. I find that while the two processes proceeded at the same time, they were not always parallel. Indeed, it is clear from some of the medical file notes that the RTWA was a cause of delay. As noted by OHS to the grievor on February 15, 2022:

Hi Joshua,
 The corporate physician reviewed the file and was made aware of the sessions you were attending and still required inpatient treatment.
 Please follow-up with your union rep to discuss next steps as until HS has received the signed Part 1 of the RTW agreement, we cannot proceed with assessing your fitness.
 CH Health Services"

25. Ultimately the amended RTWA was the means by which the lingering concerns of the CMO were addressed. This is indicated in the notes of the CMO on April 13, 2023 (see pp. 291-292, Tab 8, Company documents):

"This employee's above noted response validates this approach. To this end, in my opinion, the most appropriate first step is have this employee sign an initial two year "Step 2 Return to Work Agreement" and then monitor their compliance with the Relapse Prevention Agreement components, including the objective results of hair and urine substance testing. See "Plan" below...My

appended case note dated September 22, 2022 contains the following content: “Regarding this employee’s Alcohol Dependence (in early remission), they are to sign a Canadian Pacific two year Relapse Prevention Agreement. This is to facilitate their eventual return to a Safety Critical Position”.

26. I find the delay from January 2022 to April 2023 in order to resolve a return to work inordinately long. The concerns of the Company about ensuring the safety of the public, fellow employees and Company property, in the face of an employee with a former alcohol dependency, are legitimate. However, as I indicated in **CROA 5019**, the grievor could have been returned to work pursuant to the old RPA or the amended RTWA could have been concluded earlier.

B. At what time after January 10, 2022 and before August 15, 2022 was JB capable of performing non-Safety Critical, non-Safety Sensitive work?

Position of Parties

27. The Union argues that the grievor had provided at the very least prima facie evidence of an ability to return to work, with the January 27, 2022 FAF prepared by Dr. Rabinovitch. It notes further that even the Company had not contested the ability of the grievor to do non-Safety Critical, non-Safety Sensitive work and cites a January 5, 2022 note from the Corporate Physician to OHS confirming this.

28. The Company does not appear to argue against the ability of the grievor to do non-Safety Critical, non-Safety Sensitive work from January 2022 forward. Instead, it relies on arguments concerning proper accommodation and non-compliance, discussed in later sections, to disentitle the grievor from pay during this period.

Analysis and Decision

29. It is noteworthy that the Fitness to Work forms signed the Corporate Physician or the CMO all recognize the grievor’s ability as “Fit for Non-Safety Sensitive, non-operational, sedentary duties under direct supervision”, or “Fit for modified/alternate duties with the following restrictions” from January 6, 2022 to April 13, 2023. The only

exception in September 28, 2022, appears to be an oversight, as the September 22, 2022 note from the same CMO, only six days earlier, does recognize the grievor's Fitness to Work for modified/alternative duties (see paragraph 24, Company Brief and Tab 8, Company documents).

30. The reports from Dr. Rabinovitch and Dr. Lucknow, referred to earlier, confirm the grievor's ability to do much higher level, Safety Critical work.

31. There can be no doubt, and I find that the grievor was able to do non-Safety Critical, non-Safety Sensitive work between January 10 and August 15, 2022.

C. Did the Company meet its duty to accommodate JB?

Position of the Parties

32. The Company takes the position that it followed its Workplace Accommodation Policy throughout the period between January 10, 2022 and April 14, 2023 and explored modified/alternative duties for the grievor. It points to 3 occasions on which it explored such duties, unfortunately without success (see Tab 9, Company documents). It points out that its Montreal location is relatively small with limited administrative duties located there.

33. The Company cites **Renaud v Central Okanagan School District No. 23**, 1992, 2 SCR 970 concerning the tripartite obligations concerning reasonable accommodation. It also cites **BC (PSERC) v. BCGSEU** (1999) 3 SCR 3 (Meorin) concerning the rational connection between the safety standards followed by the Company for the safe performance of the jobs in question. Finally, it cites Arbitrator Clarke in **CROA 4503** who summarized the kind of inquiry necessary to meet the duty to accommodate:

“An Arbitrator must examine the entire process, including the assistance provided by the trade Union and the accommodated employee, plus the specific factual context, when deciding if an Employer has been sufficiently diligent in pursuing accommodation opportunities.

It is to be noted that the duty to accommodate does not include an absolute obligation to find a position. It does, however, require the Employer to demonstrate that it has made reasonable efforts to accommodate the employee up to the point of undue hardship (CROA and DR4609)."

34. The Union notes that the duty to accommodate requires an individualized assessment of the disability needs of the employee and an individualized investigation of potential accommodation measures (see **Saputo Dairy Products, 2012 HRTO 1455, AH 834, CROA 4848**). The Union argues that the Company failed to meet its duty to accommodate. It submits that the efforts of the Company to find accommodated work were minimal, and even less than the efforts found to be unreasonable in **AH 834** and **CROA 4848**.

Analysis and Decision

35. The jurisprudence is clear that it is the Company which bears the onus of establishing that it has accommodated the employee up to the point of undue hardship. In **Saputo Dairy Products**, the Tribunal held:

"The substantive component of the analysis considers the reasonableness of the accommodation offered or the respondent's reasons for not providing accommodation. It is the respondent who bears the onus demonstrating what considerations, assessments, and steps were undertaken to accommodate the employee to the point of undue hardship, see *Meorin*".

36. In **CROA 4273**, cited in **CROA 4503**, Arbitrator Picher noted the large scope of the duty to investigate alternative work as part of the employer's duty to accommodate:

I agree with counsel for the Union that it was not sufficient for the Company to determine whether there were vacant positions into which the grievor could be placed. The duty of accommodation goes further, requiring the employer to consider whether various job functions can be bundled together to create a sufficiently productive accommodated position. Additionally, the obligation of scrutiny on the part of the employer, and for that matter on the part of the Union, extends beyond the bargaining unit and can encompass managerial responsibilities or work in relation to another bargaining unit, subject only to the limitation of undue hardship.

An arbitrator must examine the entire process, including the assistance provided by the trade union and the accommodated employee, plus the specific factual context, when deciding whether undue hardship exists.

37. I am compelled to agree with the Union arguments that the Company efforts to find alternative work for the grievor were minimal at best (see Tab 9, Company documents). These efforts did not apparently extend outside of Montreal. It has not established its efforts to look for non-Safety Sensitive work, or that such work did not exist. Given the size of the Company and the large amount of administrative work required, it is difficult to believe that such sedentary work does not exist. I also agree with the Union argument that the efforts to find alternative work made in **AH 834** and **CROA 4848** were substantially greater and were still found to be inadequate.

38. Accordingly, I find that the Company onus to establish that accommodation up to the point of undue hardship was done has not been met.

D. Is JB disentitled to pay between January 13, 2022 and September 28, 2022 due to non-compliance?

Position of Parties

39. The Company argues that the grievor was non-compliant with OHS requests for information and as such, is disentitled to pay during the January 13, 2022-September 28, 2022 period.

40. The Union argues that I should disregard this argument as the Company was responsible for the delay in the return to work of the grievor.

Analysis and Decision

41. The case notes (see Tab 8, Company documents) make clear frustration on the part of both Parties. The grievor was frustrated by being out of work and the delays in

getting back to work. The Company was frustrated by the lack of information and a specific period in which the grievor refused to participate with OHS.

42. On March 16, 2022, the grievor advised that he had been advised not to speak to OHN and that he was dealing with his Union concerning the delayed return to work. This refusal resulted in a Non-Compliance notification (see pp 356-361, Tab 8, Company documents). The grievor was ultimately disciplined for this refusal, which is the subject of a grievance. However, by June 16, 2022, it is clear that the grievor is engaged in the process and providing information, although some information from Hospitals was taking time to provide (see pp 349-351, Tab 8, Company documents).

43. There is no doubt that the Supreme Court of Canada in **Renaud** places a tripartite obligation on the employee, the Company and the Union with respect to full participation on the duty to accommodate. A failure to participate can result in the loss of damages which would otherwise be awarded.

44. Here it is probable that the grievor's actions delayed the engagement of Dr. Lucknow and the production of his report. As such, the grievor is at least partially to blame for the fact that it is only by August 15, 2022, as set out above, that the Company should have found him able to perform Safety Critical work. If he had fully cooperated, it is entirely probable that Dr. Lucknow would have made his assessment much earlier.

45. The lack of cooperation was with respect to medical information concerning his alcohol dependency. This information is obviously vital before a Return to Work in a Safety Critical or Safety Sensitive role. It is difficult to understand, however, how this information would impact a sedentary non-operational, non-Safety Critical, non-Safety Sensitive role. The grievor had been provided with a Fitness to Work clearance for such work as of January 6, 2022, which was then repeatedly confirmed (see paragraph 24, Company Brief).

46. Accordingly, I do not find that the grievor's non-compliance affected his ability and right to perform such work. His pay at the lower level is therefore not affected during this period.

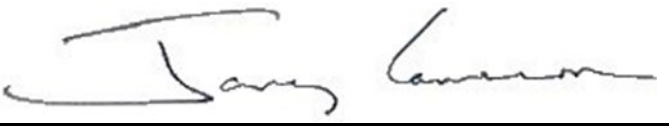
Conclusion

47. The grievor is entitled to pay and benefits from January 10, 2022 until August 14, 2022 at a rate of pay commensurate with modified duties, less mitigation.

48. The grievor is entitled to pay and benefits from August 15, 2022 until April 14, 2023 at a Conductor rate of pay, less mitigation.

49. I remain seized for any questions of interpretation or application of this Award.

November 25, 2024

A handwritten signature in black ink, appearing to read "James Cameron", is written over a solid black horizontal line.

**JAMES CAMERON
ARBITRATOR**